



CLOSER

Closer, if not near.

THE ORDINARY TUESDAY

Two point seven million people over seventy-five in Italy live on their own.

One household in ten. **92.8% of them can count on a relative. 3.2% have nobody.**

Closer is not built for the 3.2%. It is built for the **92.8%** — whose family wants to be there and is not. Work, full days, three hours of motorway.

What is missing is not love. It is the **ordinary daily contact**, where somebody would simply have noticed.

WHAT MEDICINE CANNOT REACH

We have got good at the body. Nobody is treating the silence.

We thin blood, hold cholesterol down, replace a knee, give insulin on a schedule. There is no equivalent for somebody who has not had a real conversation since Thursday. And the answer is not an interview: **they are the expert on their own past; Closer is the expert on what came after.**

WHAT TODAY GIVES THEM

No work, no travelling, reading can be hard, and the afternoon is a quiz show. **Today gives them very little to say** — and it is the past they are surest of.

WHAT THEY STILL OWN

Their past, and it is rich: a trade they were expert in, a town before it changed, a team followed since the sixties, a dance floor. **Nobody asks about any of it any more.**

WHAT CLOSER BRINGS BACK

It never tires of the sample room, and a 1960s switchboard is no trouble. It knows the history around a life, and **can say how that trade changed after they left.**

The profile carries a **window** — **1957–1998**, the years they are the authority on — and the doctrine's third job is *news from their world*. **Never test them.** Being old and being alone are a long silence, and there is no medicine aimed at it.

WHY NOW

Public payers have started buying this.

THE BUYERS ARRIVED

March 2026

Washington State covers an AI companion under Medicaid long-term services — **the first time nationwide** one has been fully reimbursed.

Six US states distribute one free. Korea has ~**14,500** in homes, mostly through local government. Italy's national telemedicine plan targets **300,000** people by June 2026.

THE USERS ARE ARRIVING

18% → 30%

AI use among US adults over fifty, in a single year.

But only **23%** of over-65s have ever used a chatbot, against 66% of under-30s. Three quarters never have.

THE BARRIER IS THE KEYBOARD

2 in 3

Italians over seventy-five are not online at all — 35.7%, rising about four points a year.

A chatbot needs an app, a login and typing. **A conversation needs none of the three.**

The category now has budget lines. Almost nobody is reaching the people who are **not online** — and they are two thirds of them.

ISTAT, Cittadini e ICT 2025 · AARP Tech Trends 2026 · Pew Research, Feb 2026 · Washington DSHS / Intuition Robotics, Mar 2026 · PNRR Missione 6

WHAT EXISTS TODAY — NOT A MOCK-UP

A person talks to Closer in a browser, or on an ordinary phone line.

The same published agent answers both, so nothing about the conversation depends on the person owning a device, a smartphone, or an app. **Which one is a choice per person:** the phone line reaches the two thirds who are not online — and a browser or an operator's own device leaves that line free to ring.

- A **setup form** that configures a companion for one specific person in about twenty minutes, filled in by someone who is not technical.
- A **diary** the person can correct out loud — it is theirs, and they can ask it what is in it.
- A **note** each conversation leaves for the next one.
- A **page their family opens** with a key. Nothing is sent to anybody; somebody opens it.

Prototype: lablab.claudiaonclaudie.com

Code: github.com/sciarada75/voice-companion — public, MIT

HOW IT IS BUILT

One agent. Two ways in. Three things kept deliberately apart.

THE PERSON

A profile generated from what the family typed in free text — who they are, what they will talk about, what their day looks like.

THE LANGUAGE

A new country is a configuration, not a rewrite. English is the default; the doctrine exists per language.

THE DOCTRINE

How the companion behaves. True for anyone, with that person's own details filled in at build time.

- Built on the **AssemblyAI Voice Agent API**. The tools are **http** tools on purpose: a client-executed tool cannot be answered on a phone call.
- The greeting **rotates** before every conversation, so it never opens the same way twice.
- **A number they say is written, never interpreted, inferred or rounded.** That stage is code, not a model — so two people running it get the same number.
- **The channel is a setting, not the product.** A browser and a phone line today, an app next — or inside the app of a partner who already has that person opening it every day. We do not have to win the home screen if somebody already has.

The conversation rules are the part that took the work.

Every rule in the doctrine is a defect heard out loud in a spoken test, and written down so it could not come back.

- It **picks one subject** rather than offering a menu. A menu is work for the person.
- Most turns **end without a question** — an open question opens a subject they then have to carry.
- It **never says where a fact came from**. Nobody wants to be read a file about themselves.
- **Two flat answers in a row and it stops**. Not every day is a day for talking.

The doctrine describes **the move, never a sentence** the model can lift — an example sentence gets recited verbatim on the first reply. That cost us two rewrites to learn.

WHERE CLOSER STOPS

Three jobs, three owners.

Closer	asks, and writes down what was said — the words unchanged, and a number they said written, never interpreted, inferred or rounded.
The family	decides what is worth seeing. The setup form asks for their day and their medicines in free text. It offers no measurements and no ranges: the moment we suggest one, we have supplied the judgement.
The operator	decides what to do about it. They have the nurses, the 24-hour desk and the responsibility.

Why, and it is not modesty. Judging a health number is a regulated medical act — it wants a notified body and a clinical evaluation, and it moves the decision to a desk with no nurse at it. And a family who trust a tool to warn them **stop watching.** Neither is a trade we will make.

THE MARKET IS THE MAJORITY, NOT THE MARGIN

Sized down, not up.

ITALY — WHERE WE PILOT

PEOPLE

WHY THIS RING

Over-75s living alone	2.7 M	ISTAT 2023. One household in ten.
... and no child within a kilometre	~1.0–1.3 M	51% of over-75s have their nearest child within 1 km. The rest are the gap.
... and living with a limitation	~550–650 k	Roughly a fifth report severe limitations in daily activities.

UNITED KINGDOM — THE PAYING PROOF

1.7–2 million people already pay £19–32 a month for a pendant alarm. That is roughly £450 M a year, spent on a button.

UNITED STATES — WHERE THE PAYER IS

16.2 million over-65s live alone. Six states already hand out an AI companion; one now reimburses it.

ISTAT 2023 · ONS / TakingCare · ACL Profile of Older Americans · NYSOFA · Washington DSHS

WHO ELSE, AND WHAT ONLY WE DO

Everything that exists is either reactive, generic, or a device.

TODAY	WHAT IT COSTS	WHERE IT STOPS
Pendant alarms Beghelli, Tunstall, Age UK	€4–£32 / month	It only speaks when it is pressed . No answer for the ordinary Tuesday.
Volunteer phone lines Auser Filo d’Argento	free, 6,000 volunteers	Inbound , office hours, capacity-bound, and nothing is written down.
AI check-in calls	\$8–30 / month	The same script for everyone . It does not know who they are.
Companion devices ElliQ	~\$900 / year, publicly funded	A device to install, in one language, in one country.

Closer is configured to **one person**, from what their family typed, in **their language**, on a **phone line they already own**. **And it moves with them:** there is no box on the wall, so the same companion answers in a spare room at Christmas, on a ward, and in the care home.

THE PRICE, AND WHY IT CAN BE FLAT

€20 a month. Talk as much as you like.

No minutes counter, no plan to choose, nothing for the family to check. About **half an hour of conversation a day** sits inside it, every day.

WHY FLAT IS POSSIBLE

The conversation is billed by **content, not by the clock**: speech-to-text, a small model, speech back. Roughly **\$0.012–0.020 a minute**.

A speech-to-speech model charges for audio in *and* out and re-reads the whole conversation each turn, so a long chat compounds. Ours does not: minute thirty costs what minute two costs.

THE COMPARISON THAT MATTERS

A carer costs **€8–12 an hour**. So €20 buys **under two hours of a person, once a month** — or **half an hour of company every single day**.

Not an alarm, which is pressed once and insures against an event. This is the ordinary day, and the ordinary day happens every day.

REVENUE, WHOLESALE

We sell **per seat to the operator**, who already owns the customer, the line and the billing.

Selling to families direct does not work: **you pay to find each one, and you lose them when the person dies or moves into care**. An operator already has them, and absorbs that.

Plus a one-off fee for the profile.

FIRST CUSTOMER: TELECARE PROVIDERS

They already own everything Closer lacks.

THEY HAVE

The exact customer, already subscribed

A phone relationship into the house

A 24-hour call centre

Distribution and billing

CLOSER HAS

A reason to speak on a day nothing is wrong

A conversation worth the person’s time

A page that says who to ring first, and why

The thing their product cannot do

Their pendant only speaks when it is pressed. Closer is **the half that speaks first** — and their call centre becomes the escalation this project deliberately does not build, because **a machine should hand a person to a person.**

THE PILOT

Fifty subscribers. Ninety days. One language.

MEASURED

Share of the words spoken by the person

Average answer length

Conversations a week, against their own baseline

Worth a call flags the desk agreed with

WHY IT IS THE RIGHT MEASURE

It rises when the conversation is working. Across twelve test conversations the thirty-day average is **13%**, the best reached **26%**, from 5–9% at the start.

Thirty-day average **7 words**; the best conversations reached **12**, from 4–5 at the start.

Compared only with themselves. **Never with a population.**

The only honest test of whether the family page is useful.

Every figure is **arithmetic, not a model’s opinion**, so two people running it get the same number.

THEN, IN ORDER

Same product each time. Different buyer.

- 1 **Comune social services.** Municipalities already run volunteer telephone rounds for over-75s living alone. A volunteer manages forty people. Closer speaks to four thousand and hands the volunteer the twelve worth a visit. It does not replace volunteers — it aims their time.
- 2 **Teleassistenza under the national telemedicine programme,** which is funded and has targets to meet by June 2026.
- 3 **Home care agencies.** The carer comes Monday, Wednesday, Friday. Closer covers the other four days, and hands the carer a page to read before going in.
- 4 **Pharmacy services.** The diary already records what was taken. A medicine routine is a pharmacy's existing relationship, not a new one.
- 5 **Care homes** — where every other product in this list loses the customer. The move is usually not a choice, and it takes the last of their habits with it. Closer is the one thing that **arrives with them, already knowing who they are.**

In Japan, care technology in nursing homes **raised care-worker employment by 28%**. It did not replace people. It kept them.

WHAT IS KEPT, AND WHERE

They are not the product.

- Recordings and transcripts are pulled to the operator, masked, and **deleted at the speech provider**.
- Secrets are removed before anything is written — only the fact that one came up is kept. The companion **refuses to be told a PIN**, itself included.
- The family page shows **no transcript**. They do not read their post. Only the people they named can open it.
- **Nothing reaches anybody on its own**. No alert, no message, no email. Somebody opens the page. That is a decision, not a missing feature.

A machine that has spoken to somebody twelve times is **not entitled to an opinion about their mind**.

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